

February 23, 2026

Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP)
and the Office of the National Coordinator for Health Information Technology (ONC)
U.S. Department of Health and Human Services

Re: RIN 0955-AA13–Request for Information: HHS Health Sector AI RFI

Dear Deputy Secretary of Health and Human Services and Assistant Secretary for Technology Policy:

Medical Information Technology, Inc. (MEDITECH) is pleased to offer a response to the Department of Health and Human Services' (HHS) Request for Information on accelerating the adoption and use of Artificial Intelligence (AI) as part of clinical care.

MEDITECH empowers healthcare organizations to expand their vision of what's possible with Expanse, the intelligent EHR platform. Around the globe, Expanse transforms care and ushers organizations into the future with AI-infused solutions, personalized workflows, next-level interoperability, and predictive analytics — all working together to drive better outcomes.

MEDITECH strongly supports the HHS initiative to accelerate AI innovation and adoption in clinical care. We recognize its potential to enhance efficiency, reduce clinician burden, and improve outcomes while maintaining patient safety and data privacy. As a leading EHR vendor, we believe a balanced federal approach is crucial—one that provides clear, risk-based guidance while minimizing undue regulatory burdens that stifle innovation. We advocate for leveraging existing EHR infrastructure as a primary vehicle for safe and effective AI deployment.

MEDITECH is aligned with and supports responses being submitted by the HIMSS Electronic Health Record (EHR) Association. We have incorporated some of the EHR Association's statements directly within our response with an attributable reference.

Below are our responses to the specific questions outlined in the RFI.

Barriers to Private Sector Innovation

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

Response:

As a leading EHR vendor, we believe that the private sector's ability to innovate with AI is primarily hindered by several intersecting issues:

- 1) Regulatory Uncertainty and Inconsistency:** The lack of a consistent, shared framework for risk management and transparency has led to a patchwork of state legislation. This forces vendors to focus on complying with multiple, diverse requirements and drains resources from actual innovation.
- 2) Lack of Risk-Tiered Approach:** Effective risk management is vital for safe AI development and establishing a consistent industry understanding of residual risk as AI technology advances. However, risk levels are not universal across all use cases. A tendency toward a uniform assessment of risk imposes a disproportionate compliance burden on low-risk applications and diverts critical resources and focus away from risk mitigation and testing needed by more novel, complex use cases.
- 3) Federal Regulation Redundancy:** Vendors are potentially subject to overlapping oversight from both ASTP/ONC and the FDA. This regulatory redundancy, often termed "double regulation," means software must adhere to both the ASTP/ONC Health IT Certification standards and FDA Medical Device standards. This compliance burden slows speed-to-market.
- 4) Access to Data:** Innovation requires access to large, diverse datasets to develop, train, and test models. Currently, the complexity of aggregating real-world data across multiple systems creates challenges with developing fully generalizable AI models.

Regulatory, Policy, and Programmatic Changes

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Response:

MEDITECH encourages HHS to prioritize the following changes:

- 1) Establish a Federal Framework:** Develop a unified, federally led framework for healthcare AI that aligns with international standards.
 - **Address State Patchwork:** Conflicting and duplicative state frameworks have emerged through a lack of federal guidance. If a vendor meets stringent federal transparency requirements (e.g., ONC HTI-1), this should satisfy state-level requirements.

- **Ensure Global Competitiveness:** Many developers operate in both the U.S. and international markets. Adopting federal standards that harmonize with global frameworks (e.g., ISO/IEC 42001:2023 and ISO/IEC 23894:2023) will position US developers for success and growth in the global market.

2) Harmonize ONC and FDA Oversight: Regulatory redundancy stifles innovation. We encourage HHS to harmonize ONC Decision Support Intervention Transparency with the FDA Pre-market Review through the following actions:

- **Transparency Reciprocity:** Compliance with the streamlined ONC Decision Support Intervention (DSI) criteria should satisfy specific FDA pre-market transparency requirements for low-to-moderate risk software.
- **“Developer-based” Reviews:** Consider FDA’s transition from “product-based” reviews to “developer-based” excellence appraisals for low-risk EHR modules. If an EHR vendor has a documented track record of safe development, their integrated AI tools should qualify for expedited clearance.
- **Expanded Enforcement Discretion:** Consider broadening enforcement discretion for AI tools that optimize workflows (e.g., auto-scribing, administrative triage, scheduling) without providing definitive diagnoses. This aligns with the deregulatory goals of HTI-5 by exempting modules that already satisfy ASTP/ONC’s Real World Testing requirements.

3) Adopt a Risk-tiered Approach: A risk-tiered framework aligns regulatory requirements with use-case risk, enabling developers to allocate resources more effectively. This approach allows innovation to begin with lower-risk applications, while providing a clear pathway for scaling to higher-risk use cases. For healthcare organizations, this approach also offers a practical roadmap for adoption, starting with low-risk, high-value use cases, while gradually building governance and monitoring capabilities.

- **Categorize Use Cases:** We encourage HHS to categorize AI use cases, differentiating them as clinical, operational, or other, to serve as a foundational step for establishing appropriate risk tiers.
- **Apply Proportional Oversight:** We encourage HHS to consider an approach that ensures regulatory requirements and oversight are proportional to potential clinical impact and safety. This allows developers to pursue a diverse portfolio of AI use cases without facing uniform compliance burdens.

- 4) Address Transparency:** Current regulation focuses on the benefit of information "to help guide decisions at the time and place of care" (see *Health Data, Technology, and Interoperability: Certification Program Updates*, [89 FR 1192, p. 619](#)). However, even with plain language descriptions, the volume of "Source Attribute" information may not be useful to the end-user.
- **Define Minimum Essential Information:** We recommend HHS support stakeholders in reaching a consensus on the minimum essential information required to ensure safe and accountable use. Focusing on essential, actionable AI documentation reduces developer burden and costly updates while ensuring critical information for AI use is available.
- 5) Create Reimbursement Incentives:** We encourage HHS to create specific CPT codes or value-based payment models that reward providers for using evidence-based AI tools that demonstrably improve efficiency, reduce costs, and improve outcomes.

Novel Legal and Implementation Issues

For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Response:

The use of AI in clinical care introduces liability and maintenance challenges that current frameworks do not address.

- 1) Establish a Liability Framework:** Liability is a complex legal issue when AI-generated insights inform clinical decision-making.
- **Duty of Care:** MEDITECH is in alignment with the EHR Association's statements regarding the need for a clearer definition of accountability for AI tools that are not regulated by FDA as medical devices: "A central issue is the allocation of liability among developers, implementers, and clinicians when AI-generated recommendations inform care decisions. Because these tools usually operate in advisory or assistive roles and do not constitute regulated medical devices, the boundaries of responsibility can be ambiguous. To support safe deployment and reduce legal uncertainty, the industry would benefit from model 'duty of care' frameworks and safe-harbor practices for documenting AI-informed clinical decisions, recognizing that the ultimate responsibility for patient care ultimately rests with the authorizing clinician. Such structures would help ensure that accountability aligns with real-world roles and capabilities while preserving clinicians' independent judgment."

- **Clarify Third-Party Liability:** Clearer rules are needed to address liability when an AI intervention relies on a third-party model that degrades due to unforeseen circumstances beyond the vendor's control.
- 2) **Leverage Transparency for Legal Clarity:** We encourage HHS to utilize the modifications proposed by HTI-5 to reorient focus toward useful transparency rather than exhaustive documentation.
- **Presumption of Diligence:** HHS should consider establishing a presumption that clinicians employing AI tools that satisfy the Decision Support Intervention (DSI) criteria—specifically, the transparency requirements outlined in 45 CFR 170.315(b)(11)—have adhered to the "standard of care."
 - **Empower Clinician Review:** Prioritizing the usability of transparency information over volume supports a clinician's ability to "independently review and understand the basis for recommendations," which constitutes the statutory prerequisite for a tool to maintain its designation as a non-device under Section 520(o) of the Cures Act.
 - **Privacy and Security Requirements:** There is uncertainty around interpretations of HIPAA privacy and security requirements when AI models are trained or updated using clinical data. HHS can play a constructive role by clarifying expectations for AI transparency and auditability, and aligning privacy guidance with real-world AI development and deployment practices.
- 3) **Ongoing Maintenance:** AI models require ongoing monitoring and maintenance to detect degraded performance caused by underlying changes in input data (known as "data drift"). This introduces a new, resource-intensive burden that creates disparity between organizations with different levels of resources. To address this, MEDITECH recommends the following:
- **Standards Version Advancement Process:** HHS should consider adopting the proposed HTI-5 deregulatory approach to Real World Testing for non-device AI. Rather than mandating prescriptive, annual testing plans, HHS could authorize a "Standards Version Advancement Process" (SVAP) methodology. HHS could permit developers of EHR-integrated AI to voluntarily adopt more contemporary interoperability standards (e.g., FHIR-based APIs) to monitor model robustness without awaiting formal federal regulatory updates.
 - **Support Equitable Capacity:** We encourage HHS to consider using grant programs and offering technical assistance to support under-resourced healthcare organizations. Funding should focus on enabling them to adopt, maintain, and evaluate AI tools. This support could also facilitate the publication of more diverse, formal evaluations of AI's impact on varying specific workflows and patient populations.

- 4) National Framework for Workforce Proficiency:** AI implementation is ineffective without a proficient workforce to deploy and maintain it. Without appropriate training, AI tools may be underutilized, misapplied, or deployed in ways that fail to achieve maximal impact. This capability includes being able to clearly explain AI applications, potential risks, and necessary precautions to patients. Investments in clinical workforce AI training and literacy at a national level are essential for fostering more equitable integration of AI across the healthcare landscape. MEDITECH encourages HHS to help set standards for baseline AI competencies and enable national workgroups to support the creation of a common set of best practices.
- 5) Clarify the Legal Status of Agentic:** The legal status of "agentic" or autonomous AI systems within EHRs remains ambiguous. HHS should finalize the HTI-5 proposal to revise the definitions of "access" and "use" in 45 CFR 171.102 to explicitly incorporate automated and AI-driven processes. This modification ensures that autonomous AI systems can retrieve and exchange Electronic Health Information (EHI), thereby facilitating the adoption of non-device AI agents for administrative and clinical support functions.

Evaluation Methods

For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

Response:

For successful and trusted AI adoption, solutions must seamlessly integrate into current clinical workflows and address real-world problems. Current evaluation approaches are fragmented, making it difficult for users and developers to consistently assess safety, effectiveness, and usability. Greater alignment around evaluation methods would support responsible adoption.

- 1) Support the Development of Standardized Metrics:** HHS could play an important convening role in encouraging clear, standardized measurement indicators tailored to distinct categories of clinical care use cases. These metrics should assess clinical effectiveness, safety, equity, and workflow impact to ensure AI tools deliver meaningful value in real-world settings. Greater consistency in evaluation methods would allow results to be more easily interpreted across settings.
- 2) Prioritize User-Centered Evaluation:** User-centered evaluation methods are vital for assessing the impact of AI on factors like cognitive burden, user

satisfaction, and perceived time savings. Evaluating human-AI interaction can help identify areas where AI functions best as a complement to human expertise, and where it may inadvertently degrade human decision-making. MEDITECH jointly published a study, [User-Centered Delivery of AI-Powered Health Care Technologies in Clinical Settings: Mixed Methods Case Study](#), that describes methods for applying user-centered research frameworks across the design and development, and post-deployment stages.¹

- 3) Encourage Structured Feedback:** Users are best positioned to identify bias or inaccuracies in AI output and surface any issues or concerns. Methods that enable users to assess and respond to AI outputs are essential for ongoing performance monitoring. Structured feedback channels can surface these insights early and support continuous improvement while reinforcing human oversight.

Private Sector Activities

How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Response:

To promote the innovative and effective use of AI in healthcare, MEDITECH proposes three key strategic recommendations:

- 1) Establish Workforce Training Programs:** To enhance AI literacy and competency across the healthcare industry, we recommend a coordinated partnership between regulatory bodies, professional organizations, and the private sector.
 - **Standardized, Role-Based:** Programs should focus on developing baseline, role-based workforce training programs and standardized credentialing mechanisms. Training modules should be customized for varying clinical and administrative roles, ensuring every member of a care team understands concepts like data quality, bias, fairness, and validity.
 - **Public Funding:** We encourage HHS to prioritize funding for these programs to ensure all healthcare professionals have access to foundational AI education. Publicly supported AI training would promote equity, improve safety and trust, and enable organizations to better achieve measurable outcomes.
- 2) Equitable Access to Accreditation/Certification:** Access to voluntary accreditation and certification processes must be kept affordable and

¹ Schreier, M., Brandt, R., Brown, H., Saensuksopa, T., Silva, C., & Vardoulakis, L. M. (2025). User-Centered Delivery of AI-Powered Health Care Technologies in Clinical Settings: Mixed Methods Case Study. *JMIR Human Factors*, 12, e76241. <https://doi.org/10.2196/76241>

available to all organizations and vendors regardless of size or financial status. Programs should align with national regulatory frameworks. Public funding through grants, subsidies, or vouchers can reduce cost barriers. To maintain a competitive market, industry-led certifications and external governance bodies should be viewed as supplemental. They must not create market conditions that impose undue financial burdens on vendors who already meet federal requirements.

- 3) Support Regulatory Sandboxes:** We encourage HHS to consider opportunities for safe experimentation through funding and support for neutral AI testing environments (sandboxes) that utilize shared evaluation frameworks and promote public knowledge dissemination. This approach would enable the generation of evidence and insights that can be used to inform policy and regulatory frameworks, while preserving industry capacity to innovate.

Enhanced Interoperability

Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Response:

To accelerate AI adoption and foster research, we encourage HHS to prioritize the following interoperability advancements:

- 1) Advance National Interoperability Frameworks:** HHS should continue to advance interoperability through the TEFCA framework, CMS-aligned data exchange networks, and standards-based APIs.
 - **Scalable AI Platforms:** Enhanced interoperability enables EHRs to function as scalable platforms for various AI-enabled workflows. This broadens market opportunities, particularly for AI tools driven by longitudinal, multi-source data, such as risk predictions, population health, and care management.
 - **Standardize Context:** Adoption of common standards further expands these markets. US Core and the International Patient Summary enable consistent clinical context, while HL7 FHIR APIs support model development and real-time AI workflows.
- 2) Improve Data Quality for AI Readiness:** High-quality, comprehensive data is essential for developing robust, generalizable, and bias-free AI tools.
 - **Support Industry Collaboratives:** We encourage HHS to continue to support industry initiatives, such as the Sequoia Project's "Taking Root Movement," which enhance AI readiness by improving data quality, metadata consistency, and patient matching, thereby reducing downstream deduplication and model bias.

3) Leverage Patient-Generated Data: Strengthening interoperability around patient access, contribution, and bidirectional exchange of patient-generated data is critical for equity.

- **Create Representative Datasets:** Bidirectional exchange would enable more representative and continuous datasets, improving AI training, evaluation, and deployment.
- **Promote Patient-Centered AI:** This shift would support the development and adoption of AI that is context-aware and effective across diverse populations and care environments.

4) Expand Data Standards for AI Outputs and Monitoring: We encourage HHS to continue to reinforce and expand data standards that support the use of AI within clinical workflows and develop standards that define how AI predictions and output are formatted in FHIR. We encourage HHS to consider ways to help build mechanisms to test, collaborate, and evolve AI use cases. As use cases move beyond pilot phases, standardized methods for users to share real-world performance data will become more crucial for ongoing monitoring, maintenance, and re-training processes.

Prioritized Research

Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Response:

We recommend HHS prioritize AI research that addresses the growing lack of privacy assurance, transparency, and trust associated with AI integration into clinical care. As automated, AI-driven data processing expands, there is a high likelihood that patients and clinicians will have limited visibility and control over how protected health information is collected, analyzed, and shared, leading to misalignment with HIPAA privacy expectations. This trust deficit represents a central barrier to widespread adoption of clinical care that needs to be prioritized to enable the safe and scalable use of AI in clinical care.

Thank you for your time and consideration. We appreciate the opportunity to share our perspective and support HHS' efforts to accelerate responsible AI adoption. We look forward to continuing to work with HHS to promote equitable adoption of innovative technologies across the healthcare landscape.

Respectfully submitted,
MEDITECH